

1. Band ligation (gold star)

WHAT YOU SEE

Rope coils tightly binding a boat hull, banner 'BAND LIGATION' with gold stars.

WHAT IT ENCODES

Acute variceal bleed = endoscopic variceal band ligation (EVL) is the definitive hemostatic therapy of choice (preferred over sclerotherapy). Perform within 12h of presentation after resuscitation. Banding both treats and prevents rebleeding.

2. Octreotide / somatostatin

WHAT YOU SEE

Crate labeled 'OCTREOTIDE' with a gold star on the dock.

WHAT IT ENCODES

Start a vasoactive splanchnic vasoconstrictor BEFORE endoscopy: octreotide (50 mcg bolus then 50 mcg/h) or somatostatin/terlipressin. Lowers portal pressure and bleeding; continue 2-5 days. Given empirically the moment a variceal bleed is suspected.

3. Prophylactic ceftriaxone (antibiotic)

WHAT YOU SEE

Crate labeled 'ANTIBIOTIC' with a gold star next to octreotide.

WHAT IT ENCODES

Prophylactic antibiotics (ceftriaxone 1 g IV daily x7d) are mandatory in any cirrhotic with GI bleed regardless of ascites: they reduce SBP, rebleeding and mortality. Give BEFORE the scope along with the vasoactive drug.

4. BEFORE the scope

WHAT YOU SEE

Banner 'BEFORE the scope' over the octreotide/antibiotic crates.

WHAT IT ENCODES

The pre-endoscopy bundle for a cirrhotic bleed: vasoactive drug (octreotide) + antibiotic (ceftriaxone), started empirically before EGD, then band ligation at endoscopy. This 'drugs first, then band' order is the high-yield sequence.

5. Portal HTN flag

WHAT YOU SEE

Boat flag 'PORTAL HTN' with an up arrow and pressure gauge.

WHAT IT ENCODES

Varices form when portal pressure (HVPG) rises >10 mmHg; bleeding risk climbs >12 mmHg. The whole variceal management chain targets lowering portal pressure (octreotide acutely, NSBB chronically, TIPS if refractory).

6. NSBB (RED X in asthma)

WHAT YOU SEE

Man pointing a 'NSBB' nail-gun with a small red X label.

WHAT IT ENCODES

Non-selective beta-blockers (propranolol, nadolol, carvedilol) lower portal pressure for primary/secondary prophylaxis (titrate to HR ~55-60). But they are CONTRAINDICATED in asthma/reactive airway disease (beta-2 blockade causes bronchospasm) and in refractory ascites.

BOARD TRAP

'NSBB (propranolol) for an asthmatic' -> wrong; non-selective beta-blockade causes bronchospasm; contraindicated in asthma.

7. Mallory-Weiss boat (Home/Observe)

WHAT YOU SEE

Blue boat with gold-star 'HOME / OBSERVE' banner.

WHAT IT ENCODES

Mallory-Weiss = longitudinal mucosal tear at the GE junction from forceful retching/vomiting (alcohol, bulimia, pregnancy). Most stop spontaneously. If NOT actively bleeding: supportive care, observe, discharge.

8. Vomit-retch-blood sequence

WHAT YOU SEE

Numbered ribbon '1 VOMIT 2 RETCH 3 BLOOD - MALLORY-WEISS' on the boat.

WHAT IT ENCODES

Classic history: repeated vomiting/retching FIRST, then hematemesis. The tear is mucosal/submucosal (vs full-thickness Boerhaave rupture, which is surgical). Bleeding is usually self-limited.

9. No PPI for non-bleeding MW (RED X)

WHAT YOU SEE

Pill bottle with red X labeled 'NO PPI' and 'NOT NEEDED'.

WHAT IT ENCODES

A non-actively-bleeding Mallory-Weiss tear does NOT need PPI therapy; management is conservative observation and antiemetics. PPI is not standard because the tear is mechanical, not acid-driven. Endoscopic therapy only if actively bleeding.

BOARD TRAP

'routine PPI for a non-bleeding Mallory-Weiss tear' -> wrong; conservative observation suffices, PPI not needed.

10. Obscure bleed: negative EGD/colon/capsule

WHAT YOU SEE

Three booths 'UPPER-SCOPE / COLONOSCOPY / VIDEO-CAPSULE' all stamped 'NORMAL', banner 'OBSCURE BLEED'.

WHAT IT ENCODES

Obscure GI bleed = bleeding persists/recurs after non-diagnostic EGD AND colonoscopy. Next step is video capsule endoscopy to survey the small bowel. If capsule localizes or is non-diagnostic with ongoing bleeding -> deep enteroscopy.

11. Deep enteroscopy (gold star)

WHAT YOU SEE

Long coiled scope with gold star labeled 'DEEP ENTEROSCOPY', banner 'OPEN-UP SCOPE / ENTEROSCOPY'.

WHAT IT ENCODES

After negative EGD, colonoscopy and video capsule, the next step is deep (double-balloon / push) enteroscopy, which both visualizes and TREATS small-bowel lesions (AVMs, tumors). It is therapeutic, unlike capsule or scans.

12. CT angiography (RED X)

WHAT YOU SEE

Red-X card 'CT ANGIOGRAPHY' on the right dock.

WHAT IT ENCODES

CT angiography is wrong as the next step for a stable obscure bleed with negative capsule: it needs active brisk bleeding ($\geq 0.3\text{--}0.5\text{ mL/min}$) and offers no therapy. Reserve for active brisk bleeding, not intermittent obscure loss.

BOARD TRAP

'CT angiography' for obscure bleed after capsule -> wrong; needs active bleeding, non-therapeutic; do deep enteroscopy.

13. Tagged RBC scan (RED X)

WHAT YOU SEE

Red-X card 'TAGGED RED-BLOOD-CELL SCAN' camera.

WHAT IT ENCODES

Tagged-RBC scintigraphy is wrong as the obscure-bleed next step: very sensitive but poorly localizing and non-therapeutic, used only for active intermittent bleeding. Deep enteroscopy is the answer here.

BOARD TRAP

'tagged-RBC scan' for obscure bleed -> wrong; localizes poorly, no therapy; enteroscopy is the next step.

14. Fecal occult blood test (RED X)

WHAT YOU SEE

Red-X card 'FECAL OCCULT BLOOD CARD'.

WHAT IT ENCODES

FOBT is wrong in an obscure overt bleeder: it only confirms blood already known to be present, adds no localization or therapy, and is a screening test, not a workup step. Proceed to deep enteroscopy.

BOARD TRAP

'fecal occult blood test' for an obscure bleed -> wrong; confirms nothing new, no localization; do enteroscopy.